

D158

Strategically Planning the Execution of a Healthcare Improvement Project

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

A nurse leader planning bedside shift report on a unit with a new coffee bar lists these factors: strong CNO support, a Joint Commission focus on handoffs, and night-shift nurses who worry report will run long and delay leaving. The unit was built in 1998. According to force field analysis, what is the most effective next move?

- A. Classify the Joint Commission focus as a restraining force.
- B. Address the night-shift time concern, for example by piloting a structured, time-limited report tool.
- C. Delay the project until the unit is renovated.
- D. Increase CNO pressure on staff to comply.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

Which factor is a driving force for implementing a new sepsis screening protocol?

- A. An EHR that cannot yet build the screening alert.
- B. Any factor mentioned by a physician.
- C. Staff concern about extra documentation time.
- D. Recent rise in sepsis deaths that leadership has made a priority.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A project team at a clinic with a new rooftop garden drafts four objectives for reducing missed diabetic eye exams. The clinic serves about 900 patients with diabetes and the team lead is a certified diabetes educator. Which objective is SMART?

- A. Ensure every patient with diabetes has an eye exam by next week.
- B. Educate all 900 patients about the rooftop garden.
- C. Improve eye exam completion as much as possible.
- D. Increase the share of patients with diabetes who complete an annual eye exam from 52% to 70% by June 30.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

On a medical unit with a strong teamwork culture and a recent magnet designation, five different nurses on three shifts missed scheduled blood glucose checks in the past month. The unit uses paper reminder sheets, and the glucometers are stored in a room at the far end of the hall. What is the best next step?

- A. Write up the five nurses and retrain them.
- B. Assume the problem is minor because the unit has a strong culture.
- C. Buy new glucometers.
- D. Do a root cause analysis of the process, including the reminder system and equipment location.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A quality leader at a five-hospital system wants to try a new early-mobility checklist. The system's CEO recently spoke about it at a national conference, and the checklist has never been used in the system. What is the best first step?

- A. Wait until a randomized trial proves it works in this system.
- B. Plan and do the change, then move directly to spreading it.
- C. Test the checklist on one unit for a short period, study the results, then adapt or spread.
- D. Roll it out to all five hospitals at once, since the CEO endorsed it.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A team planning an asthma-education project reviews ED visits by ZIP code. ZIP A, which has a new library, had 45 pediatric asthma ED visits among 3,000 children. ZIP B, home to the county fairgrounds, had 60 visits among 6,000 children. Which ZIP has the higher burden, and what is its rate?

- A. ZIP A, at 15 visits per 1,000 children.
- B. ZIP B, at 10 visits per 1,000 children.
- C. ZIP A, at 1.5 visits per 1,000 children.
- D. ZIP B, because it had 60 visits.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7. [TRANSFER — reversed stem / different variable]

A county's rate of pediatric asthma ED visits is 12 per 1,000 children, and it has 2,500 children. How many visits is that?

- A.** 300
- B.** About 208
- C.** 30
- D.** 12

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

Before introducing a new fall-risk protocol, a nurse leader at a hospital with a busy helipad shares the unit's rising fall data at staff meetings and invites nurses to discuss why change is needed. The unit recently hired six travelers. Which stage of Lewin's model is this?

- A.** Changing, because new staff were just hired.
- B.** Changing.
- C.** Unfreezing.
- D.** Refreezing.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A project plan has four tasks. Task A (3 weeks) must finish before B (4 weeks) and C (6 weeks) can start; B and C can run at the same time; D (2 weeks) starts only after both B and C finish. The project sponsor wants it done in 9 weeks and the team meets on Mondays. What is the shortest possible project duration?

- A.** 6 weeks
- B.** 11 weeks
- C.** 15 weeks
- D.** 9 weeks

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10. [TRANSFER — reversed stem / different variable]

In that same plan (A 3 → B 4 and C 6 in parallel → D 2), Task B is delayed by 1 week. What happens to the project end date?

- A.** No change; the project still takes 11 weeks.
- B.** It moves out by 1 week, to 12 weeks.
- C.** It moves out by 2 weeks.
- D.** B becomes the critical path.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A team using FMEA for a new heparin infusion process rates one failure mode, wrong concentration selected in the pump, as severity 8, occurrence 4, and detection 5. The pump vendor is based overseas and the pharmacy just hired a new director. What is the risk priority number?

- A.** 32
- B.** 160
- C.** 17
- D.** 8

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

A failure mode has an RPN of 120, a severity of 6, and a detection rating of 5. What is its occurrence rating?

- A.** 4
- B.** 20
- C.** 24
- D.** 109

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer B Concept 1

A nurse leader planning bedside shift report on a unit ~~with a new coffee bar~~ lists these factors: strong CNO support, a Joint Commission focus on handoffs, and night-shift nurses who worry report will run long and delay leaving. ~~The unit was built in 1998.~~ According to force field analysis, what is the most effective next move?

Target word: strategy **Decides:** Night-shift worry about time is a restraining force to reduce.

A	Confusion set	Regulatory focus supports the change; it is a driver.
B	Correct	Reducing a key restraining force moves change forward with less resistance.
C	Camouflage	Answers the building-age detail, which does not affect bedside report.
D	Inverted operation	Pushing drivers harder tends to raise resistance.

2. Answer D Concept 1 • transfer

Which factor is a driving force for implementing a new sepsis screening protocol?

Target word: reverse: pick a driver **Decides:** A driver pushes toward the change.

A	Adjacent category	A technical barrier is a restraining force.
B	Absolute language	Direction, not source, decides whether a force drives or restrains.
C	Inverted operation	That resists the change.
D	Correct	Data and leadership priority push toward change.

3. Answer D Concept 2

A project team at a clinic ~~with a new rooftop garden~~ drafts four objectives for reducing missed diabetic eye exams. The clinic serves about 900 patients with diabetes and ~~the team lead is a certified diabetes educator.~~ Which objective is SMART?

Target word: pick the SMART objective **Decides:** Has a metric, target, and deadline.

A	Absolute language	100% in one week is not achievable.
B	Camouflage	Not relevant to eye exam completion.
C	Dropped element	No number and no deadline.
D	Correct	Specific, measurable, achievable, relevant, and time-bound.

4. Answer D Concept 3

On a medical unit ~~with a strong teamwork culture and a recent magnet designation~~, five different nurses on three shifts missed scheduled blood glucose checks in the past month. The unit uses paper reminder sheets, and the glucometers are stored in a room at the far end of the hall. What is the best next step?

Target word: problem-solving approach **Decides:** Different staff on different shifts making the same miss points to a system cause.

A	Inverted operation	Targets individuals when the pattern points to process.
B	Camouflage	Culture and awards do not explain or fix repeated misses.
C	Dropped element	Jumps to one fix before finding the actual root causes.
D	Correct	A pattern across staff signals a system cause to fix.

5. Answer C Concept 4

A quality leader at a five-hospital system wants to try a new early-mobility checklist. ~~The system's CEO recently spoke about it at a national conference,~~ and the checklist has never been used in the system. What is the best first step?

Target word: how to start **Decides:** An untested change should be tested small first.

A	Absolute language	Improvement work uses rapid small tests, not a trial before any action.
B	Dropped element	Skips the Study step that decides whether to adopt, adapt, or abandon.
C	Correct	A small PDSA cycle limits risk and generates learning.
D	Camouflage	Endorsement does not replace testing.

6. Answer A Concept 5

A team planning an asthma-education project reviews ED visits by ZIP code. ZIP A, ~~which has a new library,~~ had 45 pediatric asthma ED visits among 3,000 children. ZIP B, ~~home to the county fairgrounds,~~ had 60 visits among 6,000 children. Which ZIP has the higher burden, and what is its rate?

Target word: higher-rate group **Decides:** Rate per 1,000: A = 15, B = 10.

A	Correct	$45 / 3,000 \times 1,000 = 15$; $60 / 6,000 \times 1,000 = 10$.
B	Inverted operation	Right rate for B, but B's rate is lower, not higher.
C	Wrong stem number	Misplaces the decimal; the rate is 15 per 1,000.
D	Dropped element	Compares counts without dividing by population.

7. Answer C Concept 5 • transfer

A county's rate of pediatric asthma ED visits is 12 per 1,000 children, and it has 2,500 children. How many visits is that?

Target word: solve for count **Decides:** Count = rate $\times$ population / 1,000.

A	Wrong stem number	Divides by 100 instead of 1,000.
B	Inverted operation	Divides 2,500 by 12.
C	Correct	$12 \times 2,500 / 1,000 = 30$.
D	Dropped element	Reports the rate as the count.

8. Answer C Concept 6

Before introducing a new fall-risk protocol, a nurse leader ~~at a hospital with a busy helipad~~ shares the unit's rising fall data at staff meetings and invites nurses to discuss why change is needed. ~~The unit recently hired six travelers.~~ Which stage of Lewin's model is this?

Target word: Lewin stage **Decides:** Building readiness and a case for change.

A	Camouflage	Hiring travelers does not decide the stage of this change.
B	Adjacent category	Changing is implementing the new protocol.
C	Correct	Sharing data to create readiness for change is unfreezing.
D	Inverted operation	Refreezing locks in a change after it is implemented.

9. Answer B Concept 7

A project plan has four tasks. Task A (3 weeks) must finish before B (4 weeks) and C (6 weeks) can start; B and C can run at the same time; D (2 weeks) starts only after both B and C finish. ~~The project sponsor wants it done in 9 weeks and the team meets on Mondays.~~ What is the shortest possible project duration?

Target word: duration from the critical path **Decides:** Longest path A-C-D = 3 + 6 + 2.

A	Wrong stem number	The longest single task, not the full path.
B	Correct	A-C-D = 11 weeks is longer than A-B-D = 9 weeks, so it sets the duration.
C	Dropped element	Adds every task and ignores that B and C run in parallel.
D	Inverted operation	Uses the shortest path; it also matches the sponsor's wish, which does not decide duration.

10. Answer A Concept 7 • transfer

In that same plan (A 3 → B 4 and C 6 in parallel → D 2), Task B is delayed by 1 week. What happens to the project end date?

Target word: effect of a delay off the critical path **Decides:** B has 2 weeks of slack (C takes 6, B takes 4).

A	Correct	B is not on the critical path and has 2 weeks of slack.
B	Absolute language	Assumes every task delay delays the project.
C	Confusion set	Confuses the slack amount with the delay.
D	Adjacent category	A-B-D grows to 10 weeks, still shorter than 11.

11. Answer B Concept 8

A team using FMEA for a new heparin infusion process rates one failure mode, wrong concentration selected in the pump, as severity 8, occurrence 4, and detection 5. ~~The pump vendor is based overseas and the pharmacy just hired a new director.~~ What is the risk priority number?

Target word: RPN **Decides:** $RPN = 8 \times 4 \times 5$.

A	Dropped element	Multiplies severity and occurrence but leaves out detection.
B	Correct	$8 \times 4 \times 5 = 160$.
C	Inverted operation	Adds the ratings instead of multiplying.
D	Adjacent category	Ranks by severity alone; RPN combines all three ratings.

12. Answer A Concept 8 • transfer

A failure mode has an RPN of 120, a severity of 6, and a detection rating of 5. What is its occurrence rating?

Target word: solve for occurrence **Decides:** Occurrence = $120 / (6 \times 5)$.

A	Correct	$120 / 30 = 4$.
B	Dropped element	Divides by severity only.
C	Confusion set	Divides by detection only.
D	Inverted operation	Subtracts the ratings from the RPN instead of dividing.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	8	
Wrong stem number	A figure copied from the prompt that is not the answer.	3	
Adjacent category	The right idea in the wrong bucket.	4	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	9	
Absolute language	Always, never, guarantee — where judgment applies.	4	
Confusion set	Every option is real; the test is exact role-matching.	3	
Camouflage	Answers a stem fact that was never the question.	5	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Force field analysis	List driving forces (pushing toward change) and restraining forces (resisting it); change is most durable when restraining forces are reduced, not just when drivers are pushed harder.	Only adding pressure through more drivers, which raises resistance.	A list of factors for and against a planned change.
2	SMART objectives	Short-term objectives must be Specific, Measurable, Achievable, Relevant, and Time-bound, naming a metric, a target, and a date.	Accepting a well-meaning goal with no number or deadline.	Several draft objectives to choose from.
3	Systems thinking and root cause	Look for causes in processes, structures, and policies (e.g., with a fishbone diagram or 5 Whys) rather than blaming individuals; fixing the system prevents recurrence.	Stopping at “the nurse forgot” and retraining one person.	A repeated problem across different staff or shifts.
4	PDSA small tests of change	Plan-Do-Study-Act tests a change on a small scale (one unit, one shift, a few patients), studies the data, then adopts, adapts, or abandons before spreading.	Launching a new process system-wide on day one.	A new idea not yet tried in this setting.
5	Rates vs. counts in population data	Compare groups with rates (events / population × a constant such as 1,000), not raw counts, because group sizes differ.	Targeting the group with the most events instead of the highest rate.	Event counts for groups of different sizes.
6	Lewin's change model	Unfreeze (create readiness and a case for change), change (implement new practices with support), refreeze (embed through policy, training, and reinforcement).	Skipping unfreezing and going straight to implementation.	An activity that builds urgency, implements, or locks in a change.
7	Critical path	The critical path is the longest sequence of dependent tasks; it sets the shortest possible project duration, and tasks on it have zero slack.	Choosing the shortest path or adding all task durations together.	Tasks with durations and dependencies.
8	FMEA risk priority number	In failure mode and effects analysis, $RPN = \text{severity} \times \text{occurrence} \times \text{detection}$; the highest RPNs are addressed first, proactively, before harm occurs.	Adding the three ratings or ranking by severity alone.	Three 1–10 ratings for a potential failure.

Course scope (public description): Strategically Planning the Execution of a Healthcare Improvement Project will help students develop the skills for systems thinking, problem-solving, and data-driven decision-making. In this course, students will plan the implementation of a healthcare improvement project by identifying people,

processes, and procedures that need to be in place for implementation. In addition, sociodemographic data on the population that may be affected by the healthcare improvement project will be analyzed to determine risks and opportunities. During this phase, students will perform an assessment of the forces that may drive or restrain implementing the project. They will also identify short-term objectives and create action plans to align to the vision, mission, and values of the organization where the project will be implemented. Students will also examine the evolution of existing policies, procedures, and processes at the systems level for the purpose of advocating change that will support a healthcare improvement project. During this course, students will plan the implementation of their healthcare improvement project through the use of sociodemographic and health data, strategic planning, and a comprehensive integration of quality and safety concepts. This course is eligible for an In Progress grade. Please see the Grading Scale Policy for more information.